PROGRAM MENU · SPEAKING & FACILITATION

Designing the experiences that carry people across boundaries.

John Vozenilek, MD: *physician, experience designer, and facilitator. Four offerings, each available as a keynote, an experiential workshop, or a module within an executive program.*

01

What Must Stay Human

Creativity, judgment, and ownership in the age of AI

02

Designing Experience That Changes Behavior

Simulation, serious games, and behavioral engineering for any organization

03

Space as a Leadership Tool

Learning spaces and experiential territory

04

Crossing the Blood–Brain Barrier

Designing partnerships between those who do and those who discover

John Vozenilek, MD
John.A.Vozenilek@jumpsimulation.org
drvoz.github.io/bespoke

▶ Watch first: medDesign podcast, from 7:40 · youtu.be/jqdyr2kO16M?t=460

OFFERING 01 · HUMAN ↔ MACHINE

What Must Stay Human

Creativity, judgment, and ownership in the age of AI

A The premise

AI is arriving in every workplace faster than the norms for using it. Leaders are rarely helped to decide which parts of the work hold their organization's judgment and creativity, or how to protect those parts while still using powerful tools.

Healthcare is a sharp test case: errors are measured in harm, and accountability can't be handed to a machine. John trains physicians to innovate with AI as a partner rather than a substitute, and this session translates those practices for any organization.

> **Perception, judgment, and decision stay with the person. Ownership is shown, not policed.**

B Participants will

01 Distinguish work AI can accelerate from the perception, judgment, and decisions that must remain with people.

02 Use an AI ledger and a team AI covenant to make human ownership visible without banning tools.

03 See why observing before solving, and recording reversals, protects creative capacity.

04 Ask sharper questions of any AI tool about consistency over time and equity across populations.

05 Leave with a plan to rehearse an AI rollout before go-live.

C In the session

01
Observe before solving
A noticing exercise with no solutions allowed.

02
Work the problem with AI
Teams use AI and keep a ledger of its contributions.

03
Debrief what stayed human
Where did judgment live? What did the tool miss?

04
Test the tool
Is it accurate over time? Fair to everyone it touches?

05
Plan the rehearsal
Rehearse an AI rollout before anyone depends on it.

06
Write the covenant
Each team defines what stays human, visibly.

D Signature stories

Rehearse first
The 3-Act-3-Debrief model: in-situ simulation to bring new technology into rural emergency departments (AHRQ-funded).
An AI rollout is a new device in the room. Rehearse it before anyone depends on it.

Name the unknown
A mobile game and a randomized trial teaching physicians to communicate diagnostic uncertainty.
As tools sound more confident, people must get better at naming what isn't known.

Test the tool
Research on whether a mortality predictor stays consistent over time and equitable across populations.
Leaders don't build models. They do ask whether a model stays true for everyone.

Prove ownership
The AI ledger and covenant of the Physician Innovator Track.
You don't have to ban the tool to keep the judgment.

E Formats

Keynote (60–90 min) · Half-day experiential workshop · Module within an immersive visit to Jump

F Ideal for

Executives leading AI adoption, health-system leaders, boards, innovation, HR, and learning leaders, and educators.

John Vozenilek, MD · John.A.Vozenilek@jumpsimulation.org

OFFERING 02 · KNOWING ↔ DOING

Designing Experience That Changes Behavior

Simulation, serious games, and behavioral engineering for any organization

A The premise

Organizations spend fortunes telling capable people what to do, then wonder why behavior doesn't change. Experience alone is a poor teacher: the events that matter most are rare, feedback is late, and there is seldom time to reflect.

Simulation was built to solve exactly that problem. Its principles, which let a resuscitation team rehearse a crisis, carry over to any organization trying to change behavior.

> **Rehearsal beats instruction. The debrief is where the learning happens.**

B Participants will

01 Apply six design principles to a behavior their organization needs to change.

02 Use debriefing as a leadership tool, not only a training technique.

03 Judge when VR, mixed reality, or games earn their cost, and when a simple model will do.

04 Build the psychological safety that honest reflection requires.

05 Choose measures that show whether behavior actually changed.

C In the session

01
Start from the behavior
Design backward from what people must do differently.

02
Make it real enough to matter
Fidelity is emotional and cognitive, not just technical.

03
Rehearse where work happens
In-situ practice surfaces real friction.

04
The debrief is the lesson
Structured reflection turns experience into change.

05
Make it safe to be honest
Psychological safety can be designed.

06
Measure the change
Mastery, observed performance, economics.

D Signature stories

Standard-setting
Chair, American Board of Medical Specialties Simulation Planning Group: simulation in board certification across all specialties.
Designed experience can become how a profession proves competence.

Behavioral engineering
An immersive behavior-change program for residents and faculty to improve learning climate, about 60 learners a year.
Culture is a set of behaviors, and behaviors can be rehearsed.

Serious games
A mobile game teaching a difficult conversation, VR intubation training, and mixed reality for sepsis education.
Choose the medium for the behavior.

Design on a budget
Published low-cost simulator designs and a U.S. simulator patent.
Realism comes from what the learner must do, not what the equipment costs.

E Formats

Keynote with live micro-simulation · Half- or full-day hands-on workshop · 1–3 day immersive program at Jump

F Ideal for

Learning and talent leaders, operations leaders in high-reliability industries, executive education programs, and educators.

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OFFERING 03 · PLACE ↔ BEHAVIOR

Space as a Leadership Tool

Learning spaces and experiential territory

A The premise

Leaders design a space once and live with it for decades. Usually the behavior it produces is discovered only after everyone moves in. Space decides who meets whom, what is visible, what can be rehearsed, and whether it feels safe to try something new.

Simulation lets leaders test a space before anyone depends on it. Realistic scenarios in a new building, workflow, or process reveal hazards and friction while they are still cheap to fix.

> **A space can be tested like a prototype. Test it before the ribbon-cutting.**

B Participants will

01 See the space they lead as a design variable for collaboration, learning, and safety.

02 Use simulation-based walkthroughs to find hazards before a space opens or a process launches.

03 Design learning and innovation spaces around the behaviors they want.

04 Apply prospective hazard analysis to a planned change in their own organization.

C In the session

01
Who meets here?
Which collisions does the space make likely?

02
What is visible?
Can people see the work and each other?

03
What can be rehearsed?
Where do people practice before it matters?

04
Is it safe to try?
Does the space invite experiment?

05
Walk it with scenarios
Simulation-based walkthroughs before opening.

06
Debrief the hazards
Users name risks first, given structure.

D Signature stories

Rehearse the building
An in-situ simulation designed to reveal latent hazards in a new clinical space before it opened.
Test the space before anyone depends on it.

Practice in place
In-situ simulation programs in operating rooms and pediatric resuscitation, including remote telehealth team members.
The best training room is often where the work happens.

Find risk early
Simulation debriefs used as prospective hazard analysis (Joint Commission Journal, 2024).
Users know a space's hazards first.

Spaces for innovation
Designed Jump (opened 2013) and its Executive Director since 2012; co-author of the $10M grant behind Peoria's Distillery Labs innovation hub.
Design an innovation space around the encounters it must produce.

E Formats

Keynote (45–60 min) · Half-day scenario-based walkthrough of the host's own space · Tour and studio at Jump

F Ideal for

Executives planning facilities or workplace change, operations and real-estate leaders, learning leaders, and innovation-district builders.

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OFFERING 04 · THOSE WHO DO ↔ THOSE WHO DISCOVER

Crossing the Blood–Brain Barrier

Designing partnerships between those who do and those who discover

A The premise

Every organization has a gap between the people who run the work and the people who could study and improve it: clinicians and academics, operations and R&D, companies and universities. Both sides want to collaborate, but they speak different languages and measure success differently.

Since 2014 John has designed the workshops, frameworks, and one-to-one sessions that guide clinicians and academics across this gap. The method has formed 69 teams and launched 308 funded projects worth $24.7M across four university partnerships.

> **Building bridges is a skill, and it can be designed.**

B Participants will

01 Map the translation gap in their own organization and who stands on each side.

02 Use a structured matching process connecting needs, skills, and data to candidate solutions.

03 Turn an operational problem into a testable hypothesis with outcomes both sides value.

04 Leave with a design for a partnership session they can run themselves.

C In the session

01
Explore mutual relevance
Find what each side needs and can offer.

02
Match skills and data to needs
Pair problems with expertise and data.

03
Frame the hypothesis
Turn a front-line hunch into a testable question.

04
Agree on KPIs and outcomes
Success in terms both sides value.

05
Hold the threads
Notice which efforts are most productive.

06
Translate and finish
Publication, pilot, or changed practice.

D Signature stories

Inception
Co-chair of a federally funded 2008 national consensus conference, which yielded seven co-authored papers in one journal issue.
Credibility comes from relevance; provenance comes from finishing.

Proof at scale
Jump ARCHES with UIUC (since 2014), replicated with UIC, Bradley, and Illinois State: 69 teams, 308 projects, $24.7M.
The method travels.

Translation
Clinical immersion courses placing bioengineering students inside clinical settings.
The fastest translation is shared observation.

Across borders
A CONFAP–Illinois project with FIOCRUZ-Amazônia, workshopped to serve the goals of Brazilian and Illinois institutions at once.
The best ideas advance several institutions at the same time.

E Formats

Half-day interactive workshop · Partnership design session for a cohort or one company–university pair · Keynote (45–60 min)

F Ideal for

Leaders of corporate–university partnerships, R&D and operations executives, executive programs on innovation ecosystems, and health systems.

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